STOP-BANG Score for Obstructive Sleep Apnea

Screens for obstructive sleep apnea.

Instructions

Note: The STOP-BANG questionnaire screens for obstructive sleep apnea (OSA) only, not central sleep apnea.

When to Use | Pearls/Pitfalls | Why Use

Ask the patient the following:

Do you snore loudly?

Louder than talking or loud enough to be heard through closed doors

No	0	**Yes**	**+1**

Do you often feel tired, fatigued, or sleepy during the daytime?

No	0	**Yes**	**+1**

Has anyone observed you stop breathing during sleep?

No	0	**Yes**	**+1**

Do you have (or are you being treated for) high blood pressure?

No	0	**Yes**	**+1**

Objective measures:

BMI

≤35 kg/m²	0
>35 kg/m²	**+1**

Age

≤50 years	0
>50 years	**+1**

Neck circumference

≤40 cm	0	**>40 cm**	**+1**

Gender

Female	0	**Male**	**+1**

ADVICE

- Clinicians may opt to modify the care of patients who are high-risk for OSA:
  - Consider reducing narcotic medication doses.

0 points

STOP-BANG

Low risk of OSA

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Next Steps | Evidence | Creator Insights

- OSA (obstructive sleep apnea) is due to a mechanical obstruction of the upper airway resulting in diminished breathing (hypopnea = narrowing) or elimination of breathing (apnea = collapse and obliteration).
- The gold standard for OSA diagnosis is polysomnography.
- CPAP (continuous positive airway pressure) is the first-line treatment of OSA and is very effective in stenting open the upper airway, but patient compliance is poor.

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Content Contributors

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